

Guide 56 — Nursing Assistant and Patient Care Technician

Controlled English working master — Version 2.0

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What this work is

Nursing assistants provide basic care and help patients or residents with activities of daily living while working within defined supervision, delegation, and safety rules. Depending on the setting, employers may use titles such as nursing assistant, nurse aide, certified nursing assistant (CNA), patient care technician (PCT), patient care assistant, hospital attendant, care aide, or similar terms.

The title **Patient Care Technician (PCT)** is not one single nationally or internationally standardized credential. Some employers use PCT for roles that overlap with nursing assistant duties; others add employer-specific skills such as phlebotomy, electrocardiography, specimen handling, mobility support, or unit-based tasks. Never assume that a PCT title authorizes a task. Verify the law, credential rules, employer policy, required training, delegation, and supervision for the exact location and setting.

This is frontline healthcare support work. It can be physically and emotionally demanding, but it can also provide a practical entry point into healthcare for people who value direct patient contact, teamwork, reliability, and service.

What you may do

Typical responsibilities can include:

- helping patients or residents with bathing, dressing, grooming, toileting, feeding, mobility, positioning, and other activities of daily living;
- measuring and recording vital signs when trained and authorized;
- observing changes in appearance, behavior, comfort, mobility, intake/output, skin condition, or other assigned indicators and reporting concerns promptly;
- assisting with safe transfers, ambulation, repositioning, and fall-prevention practices;
- making beds, maintaining a clean care environment, stocking routine supplies, and supporting unit workflow;

- transporting patients, specimens, supplies, or equipment when assigned and permitted;
- following infection-prevention, hand-hygiene, personal protective equipment, cleaning, isolation, and exposure-control procedures;
- documenting assigned care accurately and promptly according to employer policy;
- supporting respectful communication with patients, families, nurses, therapists, and other care-team members; and
- escalating changes in condition, safety concerns, suspected abuse or neglect, falls, equipment problems, medication discrepancies, or other risks through the approved chain of command.

Some PCT positions may include additional technical tasks. Those duties must be treated as **employer- and jurisdiction-specific**, not as automatic permissions attached to the job title.

What you must not do independently

A nursing assistant or PCT must not independently:

- diagnose or determine a medical condition;
- prescribe, select, change, or recommend medication treatment;
- perform an invasive or clinical procedure unless trained, authorized, delegated where required, and acting under the applicable employer and jurisdiction protocol;
- alter, invent, omit, or falsify vital signs, intake/output, observations, specimen information, care records, or safety events;
- improvise unsafe lifting, transfer, mobility, restraint, infection-control, sharps, or body-fluid practices;
- exceed role boundaries because a task appears in an online description, video, social-media post, or AI answer; or
- use generative AI as a substitute for licensed clinical judgment, patient-specific orders, emergency escalation, privacy requirements, or employer policy.

If you are unsure whether a task is permitted, stop and ask the supervising clinician or designated manager before proceeding.

Work settings

Nursing assistants and related patient-care support workers may work in:

- nursing homes and skilled nursing facilities;
- hospitals and health systems;
- rehabilitation facilities;
- assisted-living settings;

- home-care or community-care organizations;
- hospice and palliative-care settings;
- long-term care;
- specialty units; and
- other regulated healthcare environments.

Schedules may include days, nights, weekends, holidays, rotating shifts, or extended shifts. Staffing levels, scope, physical demands, and patient acuity vary widely.

Physical and emotional demands

This work often requires standing, walking, bending, reaching, repositioning patients, assisting with transfers, and working around body fluids, infection risks, distress, pain, dementia, disability, death, or family stress.

Safe body mechanics, transfer devices, fall-prevention procedures, infection controls, PPE, and employer-approved lifting practices are essential. Do not attempt an unsafe solo transfer or lift merely to save time. Ask for assistance and use required equipment.

Emotional resilience also matters. Compassion does not mean absorbing every crisis alone. Use supervision, employee support resources, peer support, debriefing, and rest practices available through the employer.

Skills employers value

Commonly valued skills include:

- dependable attendance and punctuality;
- respectful patient-centered communication;
- careful observation;
- accurate documentation;
- basic measurement and arithmetic;
- infection-prevention discipline;
- safe mobility and transfer practices;
- privacy and confidentiality awareness;
- teamwork and escalation judgment;
- emotional self-control in stressful situations;
- ability to follow instructions precisely; and
- willingness to learn employer-specific workflows and technology.

Digital skills may include electronic charting, secure messaging, scheduling, training portals, barcode or patient-identification workflows, and other employer systems. You should never share credentials or use another worker's login.

United States — entry pathway

The U.S. Bureau of Labor Statistics describes nursing assistants as workers who provide basic care and help patients with activities of daily living. Entry requirements vary by state and setting. Nursing assistants often need to complete a **state-approved education program** and pass a **state competency exam** to become licensed or certified.

A practical U.S. pathway is:

1. Review your state's nurse-aide or nursing-assistant requirements.
2. Identify state-approved training programs rather than assuming every private course is recognized.
3. Compare program tuition, exam fees, required health screening, background checks, uniforms, transportation, clinical placement, and refund terms.
4. Ask local employers whether they sponsor approved nurse-aide training or reimburse training/exam costs.
5. Complete required supervised training and competency testing.
6. Verify registry or certification status before accepting duties that require it.
7. Continue employer-specific orientation before performing unfamiliar tasks.

CMS issued current clarification in April 2026 concerning Nurse Aide Training Competency Evaluation Program (NATCEP) and Nurse Aide Competency Evaluation Program (CEP) requirements and allowable flexibilities in federally regulated nursing-home contexts. Do not assume those rules create one universal credential for every nursing assistant or PCT position in every U.S. care setting.

United States — free or lower-cost training and funding

Before paying a private provider, investigate public and employer-supported options.

CareerOneStop provides a **WIOA-Eligible Training Program Finder** that links to state directories of approved training providers. American Job Centers can help eligible job seekers understand available workforce services and training support. WIOA funding is **not guaranteed**; eligibility depends on the person, local workforce area, provider, and program.

Also ask hospitals, nursing facilities, home-care organizations, rehabilitation providers, and other healthcare employers whether they offer:

- paid nurse-aide training;
- paid onboarding or clinical orientation;
- tuition assistance;

- certification or exam reimbursement;
- uniforms or equipment support;
- transportation or shift differentials;
- internal career ladders; or
- bridge opportunities into practical nursing, registered nursing, allied health, or other roles.

Get repayment or stay-agreement terms in writing before accepting employer-funded education.

United States — pay and outlook

BLS reports the following national figures for nursing assistants and orderlies:

- Nursing assistant median annual wage, May 2024: **USD \$39,530**.
- Orderly median annual wage, May 2024: **USD \$37,700**.
- Projected combined employment growth, 2024–2034: **2%**.
- About **211,800 openings per year** are projected on average over the decade, primarily from replacement needs.

These are national occupational statistics, not guaranteed starting wages. Pay varies by state, facility type, shift, union status, experience, credential, local labor market, and employer.

Current non-government U.S. market estimate

Salary.com reports an average of approximately **USD \$34,701 per year / \$17 per hour** for Certified Nursing Assistant as of August 1, 2026, with a stated 25th–75th percentile range of approximately **USD \$31,120–\$36,431**.

This is a **non-government market estimate** based on a different methodology and title definition from BLS. Do not average it with the official BLS median. Use it only as a secondary market reference and verify local job postings before making financial decisions.

Canada — occupation and entry pathway

Government of Canada Job Bank classifies nurse aides, orderlies, and patient service associates under **NOC 33102**. These workers assist nurses, hospital staff, and physicians in the basic care of patients in hospitals, nursing homes, assisted-care facilities, and other healthcare establishments.

The current national Job Bank summary states that the occupation usually requires a college diploma, an apprenticeship of less than two years, or more than six months of on-the-job training. Exact requirements, accepted

programs, language expectations, registry rules, and employer preferences vary by province, territory, setting, and title.

Do **not** assume that a U.S. CNA credential automatically transfers to Canada. Verify the provincial or territorial rules for the location where you intend to work.

Canada — wages

Government of Canada Job Bank reports current national wage data for NOC 33102, updated November 19, 2025 with a 2023–2024 reference period:

- Low: **CAD \$19.00/hour**
- Median: **CAD \$24.00/hour**
- High: **CAD \$28.84/hour**

Use provincial and local filters before comparing an offer with the national range.

Colombia — Auxiliares en enfermería pathway

In Colombia, the closest regulated occupational pathway is broader and more formal than a generic unregulated aide title. SENA's Observatorio Laboral y Ocupacional classifies **Auxiliares en enfermería** under CNO **3311**. The occupation supports well-being, comfort, health promotion, prevention, diagnosis, treatment, rehabilitation, and palliative care **under delegation, supervision, and control of a nursing professional**, within applicable policy and regulation.

SENA Betowa currently lists an in-person titled **Enfermería** technical program of **2,640 hours**, with minimum age 16, subject to center selection, admission requirements, schedule, and seat availability. The program's clinical competencies repeatedly condition activities on delegation and health regulation.

Do not assume that a short private course creates authorization to practice as an auxiliary nursing worker. Verify the recognized training route, live SENA/Betowa availability, applicable health-sector requirements, and employer eligibility rules.

Colombia — ReTHUS

Colombia's Ministry of Health defines **ReTHUS** as the national registry for health human talent who meet requirements established for authorized practice of a health profession or occupation.

Before accepting a Colombian healthcare role, verify whether ReTHUS applies to your exact qualification and whether your registration or

authorization status is current. Do not rely solely on an employer advertisement or training provider claim.

Latin America and the Caribbean

Training, licensing, competency certification, health-worker registration, and employer rules vary by country.

OIT/Cinterfor's **Red Saber Cuidar** is a regional network focused on professionalizing care work through vocational education, competency certification, recognition of qualifications, and quality standards across Latin America and the Caribbean. In 2026, the network launched work toward a regional qualifications framework for care.

Use Red Saber Cuidar and OIT/Cinterfor as **institutional locators and policy context**, not as a license or credential. Then identify the national health authority, vocational training institution, competency body, and employer requirements in your country.

How to compare training programs

Before enrolling, verify:

- whether the program is officially recognized for the job you want;
- whether supervised clinical training is included and where it occurs;
- whether the provider has current approval from the relevant authority;
- total tuition and all additional fees;
- exam, registry, uniform, equipment, health-screening, and background-check costs;
- refund and withdrawal rules;
- schedule and transportation requirements;
- employer acceptance of the credential;
- job-placement claims and how they are measured; and
- whether public or employer-funded alternatives exist.

Avoid programs that guarantee a job or salary, hide total costs, pressure you to pay immediately, or imply legal authorization they cannot document.

Employer support and advancement

Ask employers about:

- paid orientation;
- sponsored nurse-aide or patient-care training;
- tuition assistance;
- certification reimbursement;

- shift differentials;
- mentoring or preceptorship;
- cross-training;
- internal promotion; and
- support toward practical nursing, registered nursing, phlebotomy, EKG, rehabilitation, medical assisting, or other healthcare roles where appropriate.

Do not take an extra skill course merely because it sounds useful. First confirm that the target employer actually recognizes it and that the skill is legally within the role you plan to pursue.

A practical 12-week starting plan

Weeks 1-2: Review 15-20 local nursing assistant, nurse aide, CNA, PCT, patient care assistant, and related postings. Record required credentials, schedules, pay, shift differentials, physical demands, and employer-sponsored training.

Weeks 3-4: Verify your jurisdiction's training, exam, registry, background-check, health-screening, and authorization requirements.

Weeks 5-6: Compare approved training options, including public workforce programs and employer sponsorship. Calculate the full cost, not just tuition.

Weeks 7-8: Build fundamentals in infection prevention, vital-sign concepts, communication, privacy, documentation, mobility safety, and recognizing when to escalate.

Weeks 9-10: Complete only approved supervised skills practice. Never use real patients for unsupervised practice.

Weeks 11-12: Prepare applications, interview examples, transportation plans, documentation for registry or credential checks, and questions about orientation, staffing, shift expectations, tuition support, and advancement.

Building experience honestly

Transferable experience may come from caregiving, customer service, hospitality, disability support, community service, housekeeping, food service, transport, or other work where reliability, dignity, communication, safety, and teamwork matter.

Describe what you actually did. Do not claim clinical procedures, CNA status, PCT skills, phlebotomy, EKG experience, medication responsibilities, or patient-care credentials unless you genuinely completed and were authorized for them.

Strong evidence can include:

- reliable attendance;
- respectful service under pressure;
- accurate recordkeeping;
- following safety procedures;
- helping people with mobility or daily activities within your actual role;
- escalation of problems to supervisors;
- confidentiality; and
- teamwork across shifts or departments.

Infection prevention and patient safety

Follow approved employer and jurisdiction procedures for:

- hand hygiene;
- PPE;
- isolation precautions;
- cleaning and disinfection;
- sharps and exposure prevention;
- specimen handling;
- body-fluid precautions;
- safe transfers and mobility;
- fall prevention;
- restraint rules;
- equipment checks; and
- reporting incidents or exposures.

Immediately escalate material changes in condition, abnormal observations, falls, suspected abuse or neglect, medication discrepancies, infection-control breaches, equipment problems, or other safety concerns according to training and employer protocol.

Privacy and confidentiality

Patient information may be protected by law, regulation, professional standards, and employer policy. Use only approved systems and minimum necessary access for your assigned work.

Do not:

- photograph patients or records without explicit authorization;
- discuss patient information in public areas;
- post clinical stories on social media;
- share passwords or use another worker's login;
- send protected information through personal email or messaging apps unless formally approved; or

- store patient information on personal devices unless explicitly authorized.

Using AI responsibly

AI can help with **non-sensitive** study practice, terminology review, interview preparation, generic communication exercises, or creating practice quizzes from public materials.

Do not paste patient information, photographs, clinical records, identifiers, access credentials, internal schedules, proprietary care documents, or other protected or confidential health information into public AI systems or unapproved tools.

AI must **not** replace licensed clinical judgment, patient-specific orders, emergency escalation, approved training, employer policy, or current legal and regulatory guidance. Verify clinical or regulatory information generated by AI against authoritative sources before relying on it.

Avoid scams and weak job offers

Be cautious if a recruiter, employer, or school:

- asks you to pay for a job offer;
- requests gift cards, cryptocurrency, or unusual payment methods;
- guarantees certification or employment without completing required steps;
- promises that one credential works in every state, province, or country;
- will not identify the actual employer or facility;
- asks for sensitive banking or identity data before you verify the organization;
- conducts hiring entirely through an unverified messaging account; or
- pressures you to accept unsafe duties outside your training or role.

Verify the employer, facility, regulator, and training provider independently.

When to pause before spending money

Pause enrollment when:

- the program is not clearly recognized for the occupation you want;
- the provider cannot document approval or clinical placement;
- local employers do not request the credential;
- a public or employer-sponsored route may cover the same training;
- total costs and refund terms are unclear;

- the physical demands do not fit your circumstances and no reasonable accommodation pathway is available;
- transportation or scheduling makes completion unrealistic; or
- financing terms would create debt you cannot comfortably repay.

Current sources

Official/public sources:

- U.S. Bureau of Labor Statistics — Nursing Assistants and Orderlies: [Source link](#)
- Centers for Medicare & Medicaid Services — QSO-26-08-NH: [Source link](#)
- CareerOneStop — WIOA-Eligible Training Program Finder: [Source link](#)
- CareerOneStop — American Job Center Finder: [Source link](#)
- Government of Canada Job Bank — NOC 33102 summary: [Source link](#)
- Government of Canada Job Bank — NOC 33102 wages: [Source link](#)
- SENA Observatorio Laboral y Ocupacional — Auxiliares en enfermería, CNO 3311: [Source link](#)
- SENA Observatorio — funciones/normas CNO 3311: [Source link](#)
- SENA Betowa — Enfermería, program 113535: [Source link](#)
- Ministerio de Salud y Protección Social — ReTHUS: [Source link](#)
- OIT/Cinterfor — Red Saber Cuidar: [Source link](#)
- OIT/Cinterfor — 2026 regional care qualifications initiative: [Source link](#)

Supplementary non-government salary source:

- Salary.com — Certified Nursing Assistant Salary: [Source link](#)

Source and review note

This guide uses current-source research recorded on August 19, 2026. Wages, training availability, program approvals, registry rules, delegated clinical functions, employer requirements, and funding can change. Recheck official sources and current local job postings before enrolling, paying, or accepting clinical duties.

This controlled revision was produced with AI assistance and auditable source review. It does **not** claim independent human certification, professional accreditation, certified translation, legal review, clinical approval, financial advice, or guaranteed employment outcomes.